

Introduction to Anticipatory Care

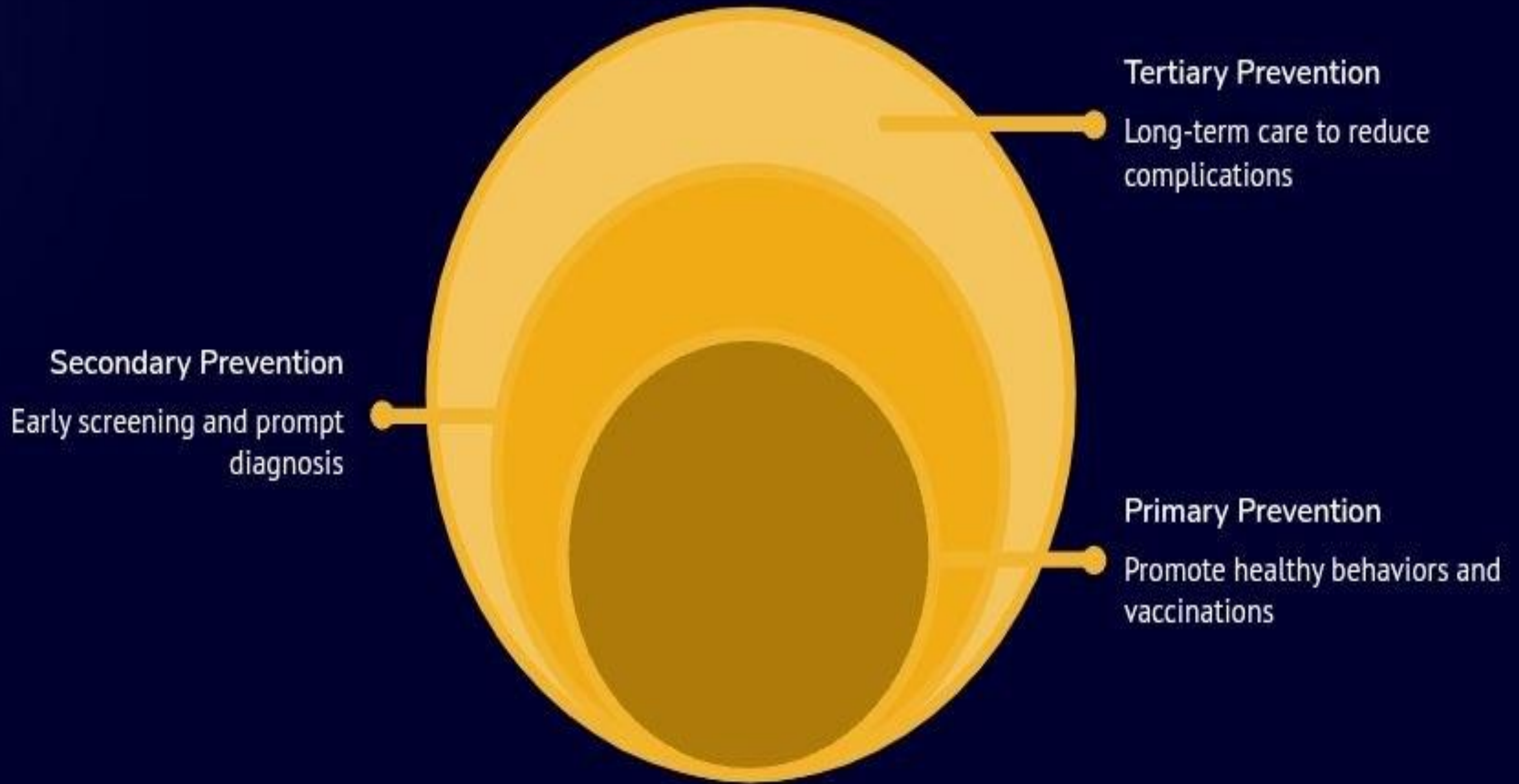
Definition: All measures aiming to promote good health and prevent or delay diseases or their complications across different age groups.

Main Goals:

- Improve quality of life.
- Reduce the burden of disability.
- Increase life expectancy.

levels of disease prevention (diagram)

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Levels of Prevention in Anticipatory Care

Primary Prevention

Preventing disease development through health education and vaccinations.

Secondary Prevention

Early diagnosis via screening and case finding.

Tertiary Prevention

Long-term monitoring to minimize the impact of complications.



Periodic Health Examination (PHE)

Purpose: Assessing risks for preventable conditions through history, physical exam, and investigations.

Key Components:

- Building a comprehensive clinical database.
- Detailed Medical, Family, and Social history.
- Occupational and Environmental assessment.
- **Anticipatory Guidance:** Counseling on diet, exercise, and substance abuse.

Anticipatory Care for Children

Goal: Ensure healthy growth and early detection of problems.

Core Pillars:

Periodic Examination

Monitoring growth and development at specific intervals (2, 4, 6, 9, 12... up to 60 months).

Immunization

Following the national compulsory vaccination schedule.

Nutritional Care

Promoting breastfeeding, proper weaning, and nutrient supplementation (Vit A, D, and Iron).

Health Education

Messages on safety, accident prevention, and treating infections like diarrhea.

Immunization in Child Anticipatory Care

1. True Contraindications to Immunization

These are the medical reasons to withhold a vaccine:

- Anaphylactic reaction to a previous dose of the vaccine.
- Seizure or fever $> 40.5^{\circ}\text{C}$ within 48 hours of a pertussis vaccine.
- True Egg Allergy (specifically for the MMR vaccine).
- Neomycin allergy (specifically for the MMR vaccine).
- Immunocompromised patients (specifically for the Oral Polio Vaccine - OPV).
- Untreated moderate to severe illness accompanied by fever.

Conditions that are NOT Contraindications

Common misconceptions where vaccines should still be administered:

Prematurity

Infants should be immunized at the usual chronological age.

Recent minor infections

Such as otitis media.

Penicillin allergy

Local reaction to a previous vaccine dose

Pregnant mother

(e.g., a child receiving MMR while the mother is pregnant).

Breast-feeding mother

(e.g., a child receiving OPV while the mother is breastfeeding).

Barriers to Vaccination (Obstacles)

Factors that hinder effective immunization coverage:

Misconceptions:

Parental: e.g., "Pertussis vaccine causes brain damage".

Physician: e.g., "Children with any fever should not be immunized".

Access to Care:

- Socioeconomic barriers.
- Geographic barriers.

Adolescent Health and Care

Definition: A transitional phase (10–19 years) marked by rapid physical, emotional, and psychological changes.

Common Challenges:



Eating disorders

Obesity/Anorexia



Psychological issues

Depression/Suicide



Risk behaviors

Smoking, substance abuse, and accidents



Role of Family Physician in Adolescence

Annual Visits: Comprehensive screening interviews and physical exams.

HEEADSSS Interview: A psychosocial assessment tool covering:

Home, Eating, Education, Activities, Drugs, Suicide, Sex, and Safety.

Prevention & Counseling:

- Discussing physical and emotional changes (Anticipatory guidance).
- Screening for BMI, Blood Pressure, and Anemia.
- Ensuring booster immunizations (Td, MMR, Hep B)

Thank you

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